



OFFICIAL

NMHS MHS POLICY

Release of Patient Information

Scope (Audience)	All NMHS Mental Health Services (MHS) Staff
Scope (Area)	All NMHS MHS Areas
Summary	Policy explains a mental health staff member's obligation to treat all patient information as confidential and provides guidelines on the circumstances and conditions where patient information may be released for certain legitimate purposes.
Risk Rating	Medium
Policy Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Comprehensive Care (Std 5) Communicating for Safety (Std 6)
Policy Sponsor	Director of Health Information Management MHSDHS
Policy Contact	Manager Area Health Information MHSDHS
Version	V2.0
Review Date	11/03/2029

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

This policy explains a mental health staff member's obligation to treat all patient information as confidential and provides guidelines on the circumstances and conditions where patient information may be released for certain legitimate purposes. This policy should be read in conjunction with:

- [MP 0015/16 WA Health Information Access, Use and Disclosure Policy](#)
- [MP 0135/20 WA Health Information Breach Policy](#)
- [NMHS Healthcare Record Access, Use and Disclosure Policy](#)
- [NMHS Information Breach Response Procedure](#)
- [NMHS Healthcare Record – Access and Document Release Policy](#)
- [Department of Health Patient Confidentiality Principles](#)
- [NMHS MHS Patient Confidentiality Guideline](#)

Schedule 1 of the [Mental Health Act 2014 \(WA\) \(MHA 2014\)](#) contains a Charter of Mental Health Care Principles and Principle 10 requires all mental health services to respect and maintain privacy and confidentiality of mental health patient information.

2. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☒	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

Adherence to this policy mitigates the risk of patient information being released to the wrong person/organisation. Failure to give patient information to the right person/organisation results in the delay of appropriate care and possible increase in harm to the patient. This policy also mitigates the risk of psychological harm or damage to the reputation of the patient through inappropriate release of sensitive information.

Adherence to this policy will reduce the risk of disciplinary action and other consequences being taken against a staff member for a breach of confidentiality. It will also reduce the risk of damage to the reputation of a staff member or health service resulting from a breach of confidentiality.

3. Policy Principles

- All staff must adhere to the principles of patient confidentiality, privacy and consent as outlined in the MP 0015/16 [Department of Health Information Access Use and Disclosure Policy](#), the [NMHS Healthcare Record Access, Use and Disclosure Policy](#) and the [Mental Health Act 2014 \(WA\)](#) and only release patient information in accordance with the procedures outlined below.
- Where staff have concerns or unsure as to whether patient information should be released or shared with other health care professionals, they should seek legal advice from

[Department of Health Legal and Legislative Services Unit](#) or through the Health Service Provider's usual processes when seeking legal advice. WA Health employees may initiate a request for legal advice by submitting the [Request for Legal Advice](#) form available via the Legal and Legislative Services webpage.

- All MHS staff must regard all information concerning patients/consumers or hospital/community service matters as confidential.
- Privacy refers to the right of an individual to control disclosure of their health information.

Whilst State Public Health Services are **not** governed by:

- [Commonwealth Privacy Act 1988](#) ;
- [Privacy and Responsible Information Sharing Act 2024](#) has not been fully enacted for Western Australia, the State Government's policy recognises the thirteen [Australian Privacy Principles](#) as the basis for policy and guideline formulation.

See [Public Sector Commissioner's Circular](#) 2014-02 (Policy framework and standards for information sharing between government agencies on the Public Sector website).

- Staff have a duty to maintain appropriate confidentiality of all patient information that they are exposed to in the course of their work.
- The duty protects information created, disclosed or acquired (directly or indirectly) by health care professionals in their professional capacity.
- Any breach of confidentiality may lead to disciplinary action; action for damages against the person (and/or his or her employer) that made the unauthorised disclosure; and disciplinary action under the health care professional's regulatory statute. Refer to the [WA Health Code of Conduct Policy](#) and [WA Health Discipline Policy](#) for further information.
- The [NMHS Information Breach Response Procedure](#) must be followed for all information breaches. Staff must also act in accordance with the [MP 0135/20 Department of Health Information Breach Policy](#) There are a number of exceptions to the duty to maintain patient confidentiality, including those found under the [Freedom of Information Act](#) (WA) (FOI Act) and under section 248 of the MHA 2014. Further exceptions can be seen where there is an overriding public interest to release the information (refer to [NMHS Managing a Public Interest Disclosure Procedure](#)) or where an order has been issued by a court of law where otherwise confidential information may be disclosed.
- Refer to the NMHS MHS Authorisation to Release/Receive Information Form (see **Appendix 1**) or the PMR3 Consent to Release/Information form (for inpatients only). The PMR3 is available for ordering in hardcopy from Print Media Group.

4. Mandatory Requirements when Releasing Information

4.1 Releasing Information

All Managers are to ensure:

- All employees, students, contractors, and volunteers are given appropriate orientation and education to both security and confidentiality requirements as per the [MP 0015/16 WA Health Information Access, Use and Disclosure Policy](#) and the [NMHS Healthcare Record Access Use and Disclosure Policy](#).
- All employees, students, contractors, and volunteers are properly informed of their responsibilities and sign either the N2 new Employment Details form or a NMHS Declaration of Confidentiality Form with the health service at the commencement of their placement/employment with a Health Service.
- All confidential material is always stored in an appropriately secure manner.

- Patient information released/disclosed in the absence of patient consent must be in accordance with the exceptions specified in these guidelines.

4.2 Consent Requirements

- Health professionals should refer to Section 4.3 of the [MP 0175/22 WA Health Consent to Treatment Policy](#) and the [WA Mental Health Act 2014 \(Part 5 Division 1\)](#) for guidance about the consent process and their obligations to seek, obtain and document valid consent from patients.
- If a patient or their designated 'proxy decision maker' expressly authorises the release of confidential information to a third party (e.g. lawyer, insurance companies, mental health advocates, health professionals outside of WA Health and/or not involved in the ongoing care of the patient) this authorisation must be documented within the clinical record. This documentation must include what may be disclosed to family/carers, what will be kept confidential, and the name of clinicians who can be contacted to discuss the patient's care and management.
- Authorisation to release information to or request information from a third party must be in writing and should be documented on either a PMR3 Authorisation to Release/Receive Information Form (for inpatients only) or on an NMHS Mental Health Authorisation to Release/Receive Information form (**see Appendix 1**)
- Additionally, if a patient or their nearest living relative is requesting release of patient information to a third party as part of a Freedom of Information application their authorisation must also be documented in the third-party consent section of the NMHS MHS FOI Application form.
- A suitable consent form may be provided by either the patient or the agent/person acting on behalf of the patient, as an alternative to the PMR3 or Authorisation to Release/Receive Information form. The consent form is only valid for a period of 12 months from the date it is signed, and consent can be withdrawn by the patient at any time (in writing). A consent form must contain the following:
 - Full name, date of birth, address and contact telephone number of the patient signing the consent to release information to the third party.
 - What information may be disclosed or received.
 - The full name, date of birth, address, contact telephone number and organisation of the third party to whom the information may be released.
 - Date that consent is being given by the patient (or their nearest living relative or guardian) or the date that consent is being withdrawn.
- Family/carers, personal support persons, guardians and nominated persons provided with confidential information about a patient must be advised by clinical staff that information provided to them in relation to a patient's care should be treated as confidential. If a patient has no family/carer/guardian, they may nominate an advocate. More information can be obtained from the [Clinicians' Practice Guide to the Mental Health Act 2014 \(Office of the Chief Psychiatrist\)](#) and the [Mental Health Advocacy Service](#).

4.3 Valid Consent

- For consent to be valid, a person must be considered competent to make a decision, that is, have the capacity to consent and understand the implications of the release of information (if any).
- All patients, including minors or older adults, are presumed to be competent in the first instance subject to obvious indicators that this is not the case, e.g. patient is not conscious, is acutely thought disordered etc.

- Issues surrounding patient consent should be carefully considered in cases where the patient is a minor or is suspected of diminished capacity secondary to cognitive impairment or serious mental illness e.g. psychosis.
- For further information on consent refer to the [NMHS Healthcare Record Access, Use and Disclosure Policy](#) and the [Department of Health Information Access Use and Disclosure Policy](#)

4.4 Minors

Refer to information on the Department of Health Legal and Legislative Services website and MHA 2014 Part 5 s. 14. The key point is that an assessment of the patient will need to be undertaken to determine whether or not they have capacity to make a decision relating to himself/ herself.

4.5 Older Adults

If an elderly patient is a voluntary patient and competent, confidential information is only discussed with family with the patient's consent as per competent adult patients.

4.6 Patients not competent to make decisions

- Health professionals should refer to Section 4.3 of the [MP 0175/22 WA Health Consent to Treatment Policy](#) for guidance about the consent process and their obligations to seek, obtain and document valid consent from patients.
- Also refer to the MHA 2014 Part 5: Decision making capacity and informed consent.
- [The Office of the Public Advocate](#) and the [Guardianship and Administration Act 1990](#) provides important information on the legal framework for three tools that can enable adults to exercise an element of control over how decisions will be made on their behalf should they ever lose the capacity to make decisions for themselves.
- The Office of the Public Advocate provides important information on the hierarchy of treatment decision makers, which includes the powers of an Advance Health Directive (AHD), Enduring Guardians, Guardians, Family members and other persons.
- An AHD sits at the top of the hierarchy of treatment decision-makers. The hierarchy sets the order in which health professionals must seek treatment decisions when treating a person with a decision-making disability, as per Sections 110ZJ and 110ZD of the Guardianship and Administration Act 1990.
- Decisions must be made in accordance with the AHD unless circumstances have changed or could not have been foreseen by the maker. Where an AHD does not exist or does not cover the treatment decision required, the health professional must obtain a decision for non-urgent treatment from the first person in the hierarchy who is 18 years of age or older, has full legal capacity and is willing and available to make a decision.
- The Department of Health also provides important information on goals of patient care. Please refer to [MH Goals of Patient Care Policy](#) and the [MH Goals of Patient Care Guideline](#).

4.7 Patient's right to refuse access to information

In accordance with [Health Service \(Information\) Regulations 2017](#), where a patient requests that verbal or written information about their care be withheld from a carer, family member, next of kin, guardian, personal support person or any other third party then that information may not be divulged to the specified person.

4.8 Power of Chief Executive of a Health Service to release or request information in certain circumstances

The [Health Services Act 2016](#) (Part 17, section 217) states:

- The chief executive of a health service provider may, in accordance with the [Health Service \(Information\) Regulations 2017](#), disclose relevant information about a patient of the health service provider to any person who, in the opinion of the chief executive, has a sufficient interest in the treatment, care, health, safety or wellbeing of the patient.
- See the associated Health Service (Information) Regulations 2017 for specific situations in which patient information may be released with the approval of the Chief Executive of a health service. However patient consent should always be sought as the first option before releasing their personal information.
- Part 26 of the MHA 2014 also gives the Chief Executive of a State authority or their delegate the power to release or receive patient information to a State authority, an interstate authority, a corresponding overseas authority or a mental health service in certain circumstances.

4.9 Recipient to be notified not to copy, modify or distribute or use patient information without consent

Where patient information is released to an individual or organisation in line with these guidelines the recipient must be notified in writing that they are not permitted to copy, release, modify, distribute or use the information in any way that identifies an individual to whom it relates. This is in accordance with Regulation 6 of the Health Service (Information) Regulations 2017.

4.10 Carers, Family, Next of Kin, Guardian, Personal Support Person or Nominated Person

- Refer to sections 263 to 302 of the MHA 2014 (Western Australia) for information on the rights of Carers, close family members, Personal Support Persons or Nominated Persons to receive verbal or written (copies of) patient information. When a clinician is in doubt as to what information can be provided verbally or in writing to one of the above carers, they should consult the three guides (linked documents) below, their Director of Medical Services or the Legal and Legislative Services Unit.
- Carers, family members, next of kin and personal support persons may request copies of specific patient information via the [Freedom of Information Act 1992](#) or MHA 2014. Current written consent will need to be provided by the patient before information can be released. If the patient does not have the capacity to consent to the release of information, their legal guardian can act as their substitute decision maker and provide written consent where deemed appropriate. Refer to the section on Consent for further information on the role of a guardian.
- Where a patient is deceased information may be released to the nearest living relative or with their written consent, to a third party. The nearest living relative's relationship must be proven before information is released, usually by production of a death certificate or a statutory declaration. Where the information being released about the deceased person may cause distress to other close living relatives e.g. the other children of a deceased parent where one sibling is requesting the information, it is advisable that the opinion of the other siblings is sought also.
- Clinicians should refer to the following documents to assist them in discussions with carers, family members, next of kin, guardians and personal support persons:

- [You Matter](#) – A guideline to support engagement with consumers, carers, communities and clinicians in health: Perth: WA Health Department 2017
- [Practical Guide for Working with Carers of People with a Mental Illness: Office of Chief Psychiatrist](#)
- [Good Practice Guidelines for Engaging with Families and Carers in Adult Mental Health Services: NMHS Mental Health Adult Program](#)

4.11 Department for Child Protection

Refer to MP 0166/21 [DoH Mandatory Reporting of Child Sexual Abuse Training Policy MP0166/21](#) and the [Guidelines for Protecting Children 2020](#)

The CEO of Department for Child Protection (DCP), an authorised delegate or an authorised officer of DCP (under Section 23 of the [Children and Community Services Act 2004 \(WA\)](#) (CCSA)) are permitted to:

- disclose relevant information to DCP or a public health authority or interested person; or
- request DCP or a public health authority or an interested person who or which holds relevant information to disclose the information to the CEO of DCP, an authorised delegate or an authorised officer of DCP.

It should be noted that:

- Only 'relevant information' may be requested. 'relevant information' is information that, in the opinion of the CEO of DCP, an authorised delegate or authorised officer is likely to be relevant to either:
 - the wellbeing of a child or a class or group of children; or
 - the performance of a function under the CCSA.
- Requests for relevant information may be made to the Department, public health authorities or 'interested persons'. An 'interested person' is a person or body that, in the opinion of the CEO of DCP, an authorised delegate or authorised officer, has a direct interest in the wellbeing of a child or a class or group of children. Therefore, an interested person may (in some circumstances) be a treating medical practitioner or other health care professional. An interested person may be requested to provide information held by him or her both in written and unwritten form.
- Compliance with a request for relevant information made by the CEO of DCP, an authorised delegate or authorised officer is voluntary. It is therefore at the discretion of the Department, public health authority or interested person to disclose confidential information when requested to do so under authority of Section 23 of the CCSA.
- No civil or criminal liability, breach of confidence or breach of professional ethics, standards or relevant regulatory statute will arise where the Department, a public authority or interested person divulges information under authority of Section 23 of the CCSA provided disclosure is made in good faith. This protection will only be available in respect of a disclosure of information to DCP. It does not provide protection for disclosures to third parties such as the police or teaching staff employed by the Department of Education.
- Confidential information may be released on the basis of a verbal request in emergency situations once the identity and authority of the person making the request has been verified (e.g., an authorised officer's identity card has been sighted and checked). A notation of the verbal request, the enquiries conducted, and any documents sighted to verify the person's authority should be recorded in the patient's medical record. Further,

the person making the request should be asked to confirm the verbal request in writing under Section 23 of the CCSA once the emergency is over.

4.12 Freedom of Information Act 1992 & s.248 of the Mental Health Act 2014

The Freedom of Information Act 1992 (WA) (FOI Act) provides the public with a legal right to request access to documents held by a government agency, subject to certain exceptions and within a specified time frame and to enable the public to ensure that personal information in documents is accurate, complete, up to date and not misleading.

Refer to the [Office of the Australian Information Commissioner website](#) – Guidelines for Agencies for further guidelines on the release of information under the FOI Act 1992.

An applicant must put their application in writing and/or complete a [NMHS FOI Application form](#). Applicants can be directed to the NMHS Mental Health Services Area Freedom of Information Office for further information and/or directed to the NMHS MHS Freedom of Information email FOI.NMHS.MH@health.wa.gov.au. Alternatively, members of the public should be directed to access the relevant information and forms via the [NMHS Freedom of Information website](#).

Under Section 248 of the MHA 2014, mental health patients/clients are entitled to inspect and be given a copy of any relevant document relating to their person, subject to certain exceptions pursuant to Section 249 of the MHA 2014.

Requests under the FOI Act and/or s.248 of the MHA 2014 should be directed, in the first instance, to the Area Freedom of Information Coordinator based at Graylands Hospital, for processing. Refer to the [MHS Release of Medical or Psychiatric Information under the Mental Health Act 2014 or Freedom of Information Act 1992 Procedure](#) for further information.

4.13 Healthcare Professionals

- Health care professionals and other individuals within a health care facility who have a legitimate therapeutic interest in the care of the patient may generally have access to the information they need to know in order to provide appropriate care and treatment. Consent to the sharing of information in these circumstances will generally be implied. If there is any doubt that the information requested is for therapeutic use, seek legal advice from Legal and Legislative Services or through the Health Service Provider's usual processes to seek advice.
- Patient information may be disclosed to health professionals outside the organisation for patient continuum of care purposes (the primary purpose) where a patient is referred to another agency for ongoing care. Where the organisation providing ongoing care is not a WA public health service patient consent must be obtained.
- Only qualified health professionals should convey clinical information directly to the requestor (providing ongoing care) by telephone. When using telephone communication, the receiver's identification must be validated prior to releasing the information. Non-clinical staff should never release patient information over the telephone.
- Students may only access clinical information if it has a direct bearing on the care they are providing or is being used to increase relevant knowledge. Students to whom access is given should be specifically made aware that the information is to remain strictly confidential, and they must sign a Patient Confidentiality statement at the commencement of their practical placement.
- Where a WA Health government organisation requests patient information and this is not part of a referral process for ongoing care, current written patient consent should always be requested from the patient.

- Where a non-government health organisation, General Practitioner, private hospital or interstate hospital requests patient information and this is not part of a referral process for ongoing care, current written patient consent should always be requested before the information is released.

4.14 Overriding Public Interest Disclosure

In matters concerning public interest disclosure there are no clearly defined rules governing the circumstances which permit the disclosure of information. Health care professionals are not under any obligation or duty to disclose information to police regarding the suspicious circumstances or injuries of their patients/clients.

Subject to a comprehensive assessment, potential justifications for the disclosure of this information to an appropriate authority relating to crime or misconduct are likely to include the following circumstances:

- Where the patient is about to commit or has committed a serious crime; or
 - Where a reasonable belief exists that a person is a risk to a child or children; or
 - Where the patient is the victim of a serious crime.
- A determination of whether a disclosure ought to be made in the public interest should be made on a case-by-case basis, assessing the particular circumstances of each case. Before information is disclosed, advice should be sought from a supervisor/manager or the Legal Services Branch, via the appropriate Clinical Director.
- Under the Children and Community Services Act 2004, there is a mandatory requirement for Medical and Nursing staff to report to DCP any suspected sexual abuse of a child and the obligation to release information under this legislation. Please refer to the Department of Health Legal and Legislative Services website.

4.15 WA Police/Coroner

The Department of Health [Patient Confidentiality Principles](#) provides advice for health workers in relation to requests from Police for patient information.

The circumstances in which confidential patient information may be disclosed to the police include:

- Where the police produce a valid search warrant or similar court document authorising the seizure of specified documents already in existence.
- Where the police provide the written consent of the patient (or other person with legal authority to consent on the patient's behalf such as the parents or other legal guardian of a patient who is a minor) and information is disclosed in accordance with the consent given.
- Where disclosure is made pursuant to a statutory protection.
- The disclosure can be justified on the basis of an overriding public interest (see 'Overriding Public Interest' in Section 5 above for further information).
- Coronial requests are processed by the Area FOI Office because the Coroner prefers to deal with a centralised body when requesting reports. The Area FOI Office has a process in place whereby they are able to track and monitor requests and seek legal advice from the State Solicitor's Office (SSO). The SSO represents the interests of NMHS at coronial inquests and reviews reports prior to submission to the Coroner's office for medico-legal purposes.

- Information relating to releasing patient information to the Coroner's Office can be found on the DoH Patient Safety Surveillance Unit website and in the Clinical Excellence Division [Review of Death Policy \(MP 0098/18\)](#).

4.16 Mental Health Advocacy Service (s.359 of the Mental Health Act)

An advocate may (subject to MHA 2014 section 359, subsection 2) inspect and copy any clinical record or other document or anything relating to a patient, whereby:

- The patient is receiving treatment involuntarily or is a mentally impaired defendant in an authorised hospital; or
- The patient is on a Community Treatment Order.

A person has the right to decline to be seen by an advocate, as well as the right to deny an advocate access to the person's clinical record.

For voluntary patients, the consent of the patient is required (section 359 (2a) MHA).

Where a clinician provides information with restricted access (see section 360 subsection 3 of the MHA) to an advocate the person in charge of a mental health service (or their delegate) must record on an identified person's medical record or file any advice given to a mental health advocate under subsection (2) about the matters referred to in subsection (2)(a) and (b).

For Digital Medical Record (DMR) sites, please note that the Mental Health Advocates have read only access to the DMR to facilitate access to the clinical record. They may only access the DMR clinical record under the conditions outlined above.

4.17 Office of the Chief Psychiatrist

In accordance with the section 521 Part 3 of the MHA 2014, while visiting a mental health service under subsection (1), the Chief Psychiatrist may do any of these things:

- inspect any part of the mental health service;
- interview any person referred to in section 515(1) who is being provided with treatment or care by the mental health service;
- require a staff member of the mental health service to do any of these things:
 - answer questions or provide information about the provision of treatment or care by the mental health service to any person referred to in section 515(1);
 - produce any medical record or other document that is held by the mental health service and relates to the treatment or care that has been or is being provided by the mental health service to any person referred to in section 515(1)
- The Chief Psychiatrist may take copies or extracts from any such record or document outlined in section 521 (3) above.
- The powers above may be exercised:
 - In relation to an involuntary patient, without restriction; but
 - In relation to any other person, only with the person's consent

4.18 Requests by Solicitors

All requests for patient information or medical reports should be referred to the NMHS Mental Health Area Freedom of Information Office. For general information on requests from solicitors refer to the [NMHS MH Patient Confidentiality Guideline](#) and the [Department of Health Patient Confidentiality Principles](#).

4.19 Research

Refer to the [MP 0162/21 Research Governance Policy](#) and the SCGHOPG Research Governance Office regarding the specific processes for conducting research within mental health services. Health professionals do not have the right to access confidential patient information for research purposes without the approval of a Human Research Ethics Committee

4.20 Quality Improvement/Auditing Activities

Patient information may be used or disclosed for a purpose directly related to the primary purpose (patient care), such as authorised quality improvement activities and authorised auditing activities conducted by a WA Health Department staff member. However written reports resulting from quality improvement and auditing activities must not store patient identifiable information. Instead, the Unit Medical Record Number or other non-identifying reference numbers must be used if required. For further advice contact the NMHS Data and Information Governance [Team](#).

4.21 Subpoenas/Summonses

Please refer to the Department of Health Legal and Legislative Services [Subpoenas and Summonses Fact Sheet](#) and the [Appearing as a Witness in Court Fact Sheet](#) for detailed information on Subpoenas and Summonses.

A subpoena/summons may require a health professional to attend a trial, produce documents and/or give evidence about those documents. Subpoenas for clinical records are to be forwarded to the Area FOI Coordinator for processing. Refer to the section on Freedom of Information & S. 248 of the Mental Health Act for the list of FOI Coordinators. This list details which FOI Coordinators process subpoenas for which sites within NMHS.

- If your service is not listed above, then the subpoena/summons is to be dealt with by your Head of Service (or delegate).
- If the patient/client is not a party to the proceedings, the patient/client should be informed of the subpoena and the place of the trial to provide the patient/client with an opportunity to object to the documents being produced if they so desire.
- Private communications between health professionals and their patient/clients are not privileged and may have to be divulged in a court. Where a judge directs a health professional to divulge information about a patient/client under oath, failure to do so can amount to contempt of court.

4.22 Australian Health Practitioners Regulation Agency (AHPRA)

An AHPRA investigator may, by written notice, request copies of patient information or a patient's medical record to assist the board in making a decision on a matter. This is in accordance with the [Health Practitioner Regulation National Law \(WA\) Act 2010](#) (Schedule 5, Part 1 - Power to obtain information). This Law (WA) requires that certain notifiable conduct be reported to AHPRA by health practitioners and their employers. Also refer to the [Notifiable and Reportable Conduct Policy MP 0125/19](#) and supporting documents for information on notifications. Requests from AHPRA should be directed to the Area FOI Coordinator. Refer to the [NMHS MHS Patient Confidentiality Guideline](#) and the [AHPRA website](#) for further information.

4.23 Transporting Clinical Information

Clinical records and clinical notes must be transported in a sealed envelope at all times. This includes transport to and from any service unit, e.g. community centres/units, day procedures centres, operating rooms, x-ray department, front reception, etc. Refer to the [NMHS MH Health Care Record Transfer and Transportation Procedure](#) and the [NMHS Healthcare Record Management Policy](#).

5. Roles and Responsibilities

All NMHS MHS staff	Are responsible for: • Having knowledge of the policy documents relevant to their roles and how to access these documents. • Complying with mandatory requirements outlined in these documents. • Assisting with policy development/reviews wherever required. • Reporting to their immediate line manager any potential non-compliance or implementation issues.
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6. Monitoring and Evaluation

Compliance with this policy is mandatory. There are legislative obligations relating to this policy where a statutory penalty may apply for non-compliance.

All breaches of patient confidentiality should be logged in DATIX CIMS by the line manager of the person/s involved in a breach of confidentiality.

The number of Information Breach Notification forms sent to the NMHS Business Intelligence Unit will determine compliance rate.

Yearly KPI figures can be generated by the NMHS Business Intelligence Unit to monitor compliance with this policy. The expected rate of compliance is 100%.

This policy will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three (3) years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

7. Definitions

Terms	Definitions
Confidentiality	The treatment of information that an individual has disclosed in a relationship of trust and with the expectation that it will not be used or divulged to others in ways that inconsistent with the understanding of the original disclosure, without permission.
Information security	The practice of defending information/data from unauthorised access, use, disclosure, disruption, modification, perusal, inspection, recording or destruction.

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms

8. Related Documents

Legislation	• <u>Privacy and Responsible Sharing Act 2024</u> • <u>Mental Health Act 2014 (WA)</u> • <u>Carers Recognition Act 2004</u> • <u>Children and Community Services Act 2004</u> • <u>Freedom of Information Act 1992</u> • <u>Guardianship and Administration Act 1990</u> • <u>Health Practitioner Regulation National Law (WA) Act 2010</u> • <u>Health Services (Information) Regulations 2017</u> • <u>Privacy Act 1988</u> • <u>Health Practitioner Regulation National Law (WA) Act 2010</u>
WA Health mandatory policies	• <u>WA Health Acceptable Use of Information and Communication Technology Policy (MP 0066/17)</u> • <u>WA Health Information Access, Use and Disclosure Policy (MP 0015/16)</u> • <u>WA Health Information Breach Policy (MP 0135/20)</u> • <u>Department of Health Patient Confidentiality Principles</u> • <u>Aboriginal Data Governance Policy (MP 0190/25)</u>
NMHS Policy documents	• <u>NMHS Health Record Management Policy</u> • <u>NMHS Information Breach Response Procedure</u> • <u>NMHS My Health Record (MHR): System Control, Administration, Access, Upload and Security Procedure</u> • <u>NMHS Healthcare Record – Access and Document Release Policy</u> • <u>NMHS Authorisations, Delegations and Decision Making Schedule</u>
NMHS MHS Policy documents	• <u>NMHS MHS Patient Confidentiality Guideline</u> • <u>NMHS MHS Health Care Record Transfer and Transportation Procedure</u> • <u>NMHS MHS Patient Information – Electronic Transmission of Guideline</u> • <u>NMHS MHS Clinical Records Management and Storage Procedure</u>

9. Document Control

Document Version Control			
Date	Version	Author	Notes
11/05/2022	1.0	Area Health Information Manager	Initial endorsement; this policy provides specific release of patient information processes for mental health including processes for AHPRA, Area FOI, Police, Coroner, consent to release, rights of patients to refuse access to information and the rights of the Chief Executive of a Health

			Service to release information in certain circumstances. MH Consent to Release/Receive information added, template and links updated.
11/03/2026	2.0	Manager, Area Health Information	Scheduled review, updated overarching legislation and policy information, consent requirements refreshed, inclusion of MHS Release of Medical or Psychiatric Information under the Mental Health Act 2014 or Freedom of Information Act 1992 Procedure, DMR information added, OCP information updated, Research Ethics Governance refreshed, list of FOI coordinators removed.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Appendix 1: MHS Authority to Release/Receive Information Form



Government of Western Australia
North Metropolitan Health Service
Mental Health, Public Health and Dental Services

APPENDIX 1

AUTHORITY TO RELEASE / RECEIVE INFORMATION

TO: _____

_____ (NAME AND ADDRESS OF HEALTH SERVICE)

I, _____, of
FULL NAME OF PATIENT/PERSON GIVING AUTHORITY PATIENT/PERSON DATE OF BIRTH

_____ FULL ADDRESS OF PATIENT GIVING AUTHORITY

HEREBY AUTHORISE any authorised officer from the above named health service to (circle whichever is applicable – a or b):

- a) Release information to a carer/advocate/guardian/relative and/or other professional agencies who are involved in my care; or
- b) Obtain information from a carer/advocate/guardian/relative and/or other professional agencies who are involved in my care.

Detail the specific information to be released or obtained e.g. a medical report, witness statement or copies of my medical record, test results, discharge summaries and other documentation concerning my treatment at the healthcare facility:

PARTY TO WHOM INFORMATION IS TO BE RELEASED TO OR OBTAINED FROM:

FULL NAME: _____

ADDRESS: _____

TELEPHONE: _____ EMAIL: _____

PATIENT/PERSON GIVING CONSENT:

The purpose of this consent form is to ensure Duty of Confidence and Continuity of Care.
This consent form will remain valid for a period of twelve months from the date signed.
Expiry date of consent: _____

I understand that I can withdraw consent at any time within the twelve-month period.

SIGNATURE: _____

PRINTED NAME: _____

DATED: _____

WITNESS SIGNATURE: _____

WITNESS PRINTED NAME: _____

CONSENT WITHDRAWN (SIGNATURE): _____ DATE: _____